



OFFICIAL

NMHS MHS PROCEDURE

Search of Person, Property, Environment and Seizure of Property

Scope (Audience)	NMHS Mental Health Services (MHS) Staff and Security Staff
Scope (Area)	All NMHS MHS Inpatient Areas (Graylands Hospital, Sir Charles Gairdner Hospital, Osborne Lodge Selby Lodge and State Forensics Mental Health Service (SFMHS))
Summary	This procedure is designed to balance the organisational duty of care to provide a safe environment with the maintenance of human dignity and ensure that legislative requirements are upheld during the process of searching a person, property, or the patient environment in accordance with the Mental Health Act 2014 (MHA 2014)
Risk Rating	High
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2)
Policy Sponsor	Director of Medical Services MHSDHS
Policy Contact	Operational Co Directors Adult Inpatient MHSDHS
Version	4.0
Effective Date	25 November 2025
Review Date	25 November 2026

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

North Metropolitan Health Service Mental Health Services (NMHS MHS) is committed to providing a safe and therapeutic environment for all.

This procedure is designed to balance the organisational duty of care to provide a safe environment with the maintenance of human dignity and ensure that legislative requirements are upheld during the process of searching a person, property, or the patient environment.

2. Background

In order to provide a safe and therapeutic environment for mental health patients it is sometimes necessary to search patients and remove unsafe, dangerous or illegal items.

Undertaking a search of a person or their property may cause distress, create mistrust, or trigger memories of previous trauma that are intensified in the context of a MHS environment

This procedure aligns with the following documents:

- [Mental Health Act 2014 \(MHA 2014\)](#)
- [Clinicians Practice Guide to the Mental Health Act 2014](#)
- [MP 0175/22 WA Health Consent to Treatment Policy](#)
- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)
- [MHS Management of Restrictive Practices Policy and Procedure](#)
- [MHSDHS Mandatory Training Policy](#)

Staff undertaking a search must take active steps to ensure that trauma informed care principles and the dignity of the individual are maintained by approaching the situation in a collaborative and supportive way that is balanced with the need to maintain safety for all involved.

MH inpatient areas must identify items that are prohibited in MHS, and that this information must be made available for patients and staff and must be reviewed annually.

During the search of a person, property, environment, and seizure of property NMHS MHS promotes the use of a risk assessment approach inclusive of the use of positive risk taking, rather than a punitive or restrictive approach. The risk assessment approach that is to be utilised when initiating the procedures of search and seizure of patient personal property is one that balances both the likelihood of harm occurring, and the consequences of the potential harm caused, versus the impact for the individual in not being able to access their personal property.

It is acknowledged that not all risk of personal injury or harm can be mitigated through the process of searching, or routine removal of items that could be used to cause harm to self or others. Staff need to be aware of not only the procedures of searching and removing items of concern but other interventions such as assessment, monitoring and observation that mitigate risk. Searching and removal of certain items is particularly prominent to mitigate the following risks:

- Weaponisation of items to perform harm to others or use in self-protection.
- Using items to engage in self-harming activities.
- Using items to attempt or complete suicidal acts.
- Harmful effects of inhalants, intoxicants and other chemicals.

3. Risk

Non-adherence to this procedure could result in:

- Patients becoming distressed, or traumatised if searched in an undignified manner.
- A breakdown in the trust relationship between patients, their families and clinicians.
- Patients, their families, and the staff of the service exposed to an unsafe environment due to items that are inappropriate for the clinical setting.
- Clinicians and the service breaching the legislative requirements of the *MHA 2014*.

4. Removal of Items

4.1 Weaponisation of Items

It is not possible to remove all items from clinical environments that may be used to cause harm to self or others. If person feels threatened or is assaultive for any reason, they can use their body to cause harm (i.e. punching, kicking, jumping, choking).

Careful environmental management of clinical areas is undertaken to reduce risk and includes a number of processes that are intended to remove, mitigate or minimise risk especially for services that care for patients with identified risks to others. Mitigation strategies to address the risk of physically assaultive behaviour include:

- rapid and thorough assessment and identification of risks (refer to [MH Management of Clinical Alerts Policy](#));
- building of therapeutic rapport and trust (refer to [MHSDHS Mandatory Training Policy](#));
- reducing opportunities by maintaining spatial awareness and being mindful of proximity;
- ensuring staff do not work in isolation;
- use of Personal Duress Alarms (PDAs) (refer to [MHS Duress Alarm Systems Procedure](#));
- increased clinical observations (refer to [NMHS MH Clinical Observation Levels Procedure](#)).

4.2 Self-Injury

In inpatient settings, removing items that may be used to self-harm or engage in suicidal acts may be necessary when patients are experiencing high levels of distress and are unable to participate in safety planning or to commit to managing their own safety. It may be possible to return some of these items throughout the admission depending on the outcome of assessment and risk. The need to remove these items for the entire duration of an admission may discourage the use of positive coping strategies or behaviours. Patients need to be provided with the opportunity to practice alternative coping skills when experiencing distress or despair, including around objects that will be within their normal everyday environment.

While the risk of harm to self needs to be assessed and mitigation strategies put in place, the main clinical interventions that should be considered to manage risk of harm to self include:

- Increased clinical observation and engagement.
- Building of strong therapeutic relationships.
- Identification of triggering events.
- Promotion, teaching, and facilitation of alternative positive coping strategies.

4.3 Intoxicants

Patient use of legal or illegal intoxicants decreases impulse control and judgement, which in turn greatly heightens the risk of self-harm or assaultive behaviour.

Any patient who is found in a state of intoxication or in possession of intoxicants should be immediately clinically assessed by the most senior nurse on duty and/or the Medical Officer to determine the most appropriate clinical interventions.

5. Search Process

5.1 Prior to search

The service will ensure the following:

- Uphold the patient's right to be informed and involved in all searches.
- Acknowledge that for Aboriginal patients, consideration is given to contacting the local Aboriginal Health Liaison Officer for support and assistance.
- Personal searches must be undertaken by persons of the same gender as the patient and managers will ensure the availability of staff of appropriate genders to perform searches.
- Personal searches are conducted in a suitable and private location away from other patients, staff and visitors.
- Provide substitute clothing for patients to wear in the event searches involve removal of clothing
- Provide access to information leaflets on legal rights under *MHA 2014*.

5.2 Requesting a search

Staff are to:

- Introduce themselves, including the position they hold.
- Explain what is happening and what is required and seek the co-operation of the patient
- Take into consideration culture, language and gender preferences and the requirements of the [NMHS Chaperone Policy](#).
- Use interpreter services or other communication aids as required.
- Verbally inform patients in advance, of situations when they will be requested to agree to be searched as a routine safety precaution in the presence of a chaperone (e.g. informing patients that they will be requested to submit to a search on their return from leave; ground access).
- Explain the rationale for the search request to the patient, and if appropriate the personal support person.
- Approach the patient in a non-confrontational manner.
- Whenever possible, a request for a patient to consent to a search should not occur in a public forum.
- Ensure appropriate privacy is provided during the search process and that the person is always treated with dignity and respect during the process.
- Offer a staff member (ideally known to the patient, and considering gender and cultural sensitivities) not performing the procedure of the search as a chaperone ([NMHS Chaperone Policy](#))
- Request the patient to consent to the search.
- Request cooperation from the patient even if the search is being performed without consent.
- Request the patient to be actively involved in the search as far as practicable.
- Provide the patient with reassurance and encouragement during the process.

- Offer and provide verbal explanation of patient rights under the *MHA 2014* and provide available information leaflets.

5.3 Search with consent

If expressed verbal consent to a search is given by a patient, then the procedure of the search can be completed without utilising the powers of the *MHA 2014*.

The patient must be informed that they can withdraw consent at any time.

The absence of refusal to be searched is not considered consent to the search. Staff should gain verbal confirmation of consent. If staff have concerns regarding the patient's capacity to consent or understand the request, they need to first discuss this with the Shift Coordinator who will decide to either escalate by:

- Discussing capacity to consent and rationale with Senior Registered Nurse.
- Discussing capacity to consent and rationale with treating Psychiatrist (or delegate).
- Proceeding with the search on the advice of the Senior Registered Nurse/Psychiatrist (or delegate) without the expressed consent of the patient.

Searches with consent are to be documented in the progress notes, with accompanying risk assessment if conducted outside of routine. When documenting the search in the progress notes staff must include:

- The process of consent being given.
- The risk assessment that led to the request to conduct a search.
- Describe the search using the domains described in this policy (rationale, location, consent).

5.4 Search without Consent

The powers vested to MHS in Part 11, Division 2, s160-165 of the *MHA 2014* allow for searches to be conducted without consent for anyone presenting for treatment, or under the care of the MHS regardless of legal status (voluntary or involuntary).

Regardless of this the use of this power to search a voluntary patient without consent should be carefully considered. If a voluntary patient does not consent to a search, a further assessment and review of the situation and the patient should be undertaken and include the admitting Medical Officer. This assessment and review could include offering the patient discharge as an option. The use of referral for examination or detainment of a voluntary patient at an authorised hospital should only be considered if the patient's clinical presentation meets the criteria outlined in the *MHA 2014*.

Regardless of the refusal to give consent to a search, a patient is to be informed if the search will proceed under the powers of the *MHA 2014*. The patient should be offered a chaperone and receive an explanation regarding the rationale for the search and the preference for their consent to be given. They should be asked to cooperate with the search to reduce any risk of harm to the patient or staff involved and physical force should be avoided. Bodily restraint must only be used if necessary, reasonable, and duly authorised under the provisions of the *MHA 2014*.

Searches without the patient's consent must be documented in both the progress notes (with particulars as detailed in section of searching with consent) and the authorised *MHA 2014 Form 8A*. A clinical incident report is to be created via [DatixCIMS](#). The original Form 8A is to be placed in the patient's medical record which copies to be supplied to:

- The patient.
- The patient's guardian, or nominated person (if indicated).
- The Senior Registered Nurse – will attach a scanned copy to the [DatixCIMS](#) incident record and table the incident for review in accordance with the endorsed process for review in the service.

For sites using Digital Medical Records (DMR), store the original Form 8A in the patient's Work in Progress file for later scanning in the DMR. Once scanned, the Form 8A will appear in the Legal Folder for the patients DMR.

A copy of the form to be supplied to:

- The patient.
- The patient's guardian or nominated person (if indicated).
- The Senior Registered Nurse – will attach a scanned copy to the [DatixCIMS](#) incident record and table the incident for review in accordance with the endorsed process for review in the service.

5.5 Visitors – seeking consent and searching of

A visitor may not be searched without their consent. If the visitor refuses to consent to a search, staff can consider the following:

- Observed/supervised visits with the patient
- Restricted visitor access to the ward (see [MH Visitors, Relatives, Carers, Person Support Persons and Friends Procedure](#))
- Request the visitor leave the premises
- Consider involved WA Police Force (WAPOL) if visitor behaviour necessitates this
- If there is concern about visitation of any of the above actions have been taken, this should be documented in the medical record of the patient visited. If a patient is not involved, an email should be sent to relevant executive, and SRN/RM of the clinical area involved with incident details. Consider also notifying Security to relevant information sharing.

5.6 Safety during a search

- A **minimum of two (2) clinical staff must be present on any occasion where a search is undertaken.**
- All searches will be completed in the presence of at least two clinical staff including searches of ward areas and bedrooms that are conducted when the patient is not present.
- If a search is undertaken when the patient is not present, the senior clinical staff member holds responsibilities for ensuring the process is followed, including verification and listing of property that is removed.
- Clinical staff can ask security staff to assist in carrying out a search in the presence of a clinician.
- **Gloves must be worn when searching patient's property.** High risk areas should consider having Kevlar gloves available for searching as they offer better protection against injury from sharp objects.
- Wherever practicable, the patient should be present throughout any search of their belongings and take an active role in the process (i.e. emptying pockets, opening bags etc.). Searches undertaken without the patient present should be considered outside of normal process.

- Any search of a person will comply with the requirements for a support person to act as a chaperone as set out in the requirement during Intimate Area Examinations under the [NMHS Chaperone Policy](#).
- When conducting a search that involves touching a person or their property, staff will be mindful of the risk of cross contamination of infectious disease or infestations, potential contact with bodily fluids, potential contact with hazardous items.
- Staff are to utilise the PPE appropriate to the level of risk identified (either of injury or indicated by infection control).

Note: Kevlar gloves that minimise the risk of injury from needle sticks are provided in clinical areas.

5.7 Patient Involvement in process of search

Regardless of the type of search, the patient should be involved as far as practicable in the search process. The involvement of the patient in the search process helps in maintaining dignity and safety of the patient, reduces risk of staff being injured by hazardous items, and ensures that the process is conducted in a manner that is respectful.

The level of patient involvement will depend on the clinical presentation, personal wishes of the patient involved, and the type of the search being conducted. Examples of how a patient can be involved in search procedures include:

- removal of clothing;
- running of own hands through hair;
- opening and unpacking of bags;
- turning out pockets;
- asking the patient to identify and handle/move specific items (i.e. patient self-identifying that a bag contains uncapped used syringes and transferring these items into a sharps container).

6. Types of Search

There are many factors to consider when performing and describing a search. These factors will also affect how the search is conducted and documented.

The following domains are to be used to describe a search:

- Rationale
- Area or location
- Consent

6.1 Routine searches

As a part of ongoing procedures to maintain safety of each service, some searches are conducted at routine intervals, or at routine points of the patient journey as identified in this procedure.

Each service or ward area is required to consider and publish a list of times that routine searches should be conducted within the local area. This will be influenced by the nature of the service (inpatient, community, authorised or non-authorised).

It is essential that staff clearly inform patients in advance if a search is being requested as part of routine procedures in the service.

For all inpatient services the following points of a patient journey must be considered a point of routine search:

- **At admission** - prior to entering the ward area, a search of the person and all patient property will be conducted. Patient property may be held in a secure area until a search can be practicably conducted. As all property must be searched the patient should be given alternative clothing to wear while personal clothing is searched.
- **Prior to transfer to another MH unit or discharge** – transfer of patients to another part of the service requires a nurse escort to ensure appropriate handover of clinical care. Prior to transfer (including ward to ward, and change of service) a search of person, property, and patient environment will be conducted.

At discharge a search of the patient environment will be conducted. This is partly to identify any items of risk but is also to ensure that the patient doesn't leave behind any personal property.

- **Daily cleaning of shared areas** –all furniture should be checked for items that could have been discarded or secreted by people in the environment. If staff find items of food these are to be discarded appropriately. If staff find items that are considered unsafe within the ward area this is to be immediately reported to the Shift Coordinator for immediate assessment of the situation and planning of interventions. If the items are considered to constitute a risk, they will be reported as a clinical incident via [Datix CIMS](#).

For secure authorised inpatient areas, the following points of a patient journey should also be considered a point of routine search:

- **Upon return from escorted leave or ground access** –escorting staff will provide continuous observation including ensuring return of any item that is considered safe on ground access but not allowed within the inpatient ward.
- **Upon return from unescorted leave or ground access** – following patient return to the unit a search of the person will be conducted. Any additional property bought whilst out may be held in a secure area until a thorough search is conducted.

6.2 Outside of routine search

A search outside of the local service/ward searches can be conducted on any of the areas/locations listed.

A risk assessment approach that balances both the likelihood of harm occurring, and the consequences of the potential harm caused versus the risk of loss of patient dignity, functionality and personal responsibility is to be utilised. Searching outside of routine must be based on risk assessment and must be clearly documented in the progress notes.

Some situations that aren't planned or routine interventions will always trigger a search process. These include:

- Search of the person prior to seclusion.
- Search of the shared environment, and potentially of multiple persons when there is an identified loss of cutlery or controlled item.

6.3 Person

A search of the person can include:

- Scanning the person with an electronic or mechanical device, whether handheld or not i.e. a metal detector).
- Removal and searching of the person's outer clothing, including headwear, gloves, footwear, coats or jackets. Inner clothing or underwear cannot be removed for the sole purpose of searching.
- A physical frisk search – a quick and methodical running of the hands over the outside of the person's clothing to detect larger concealed items that could be hazardous. A frisk search can be very limited in detecting smaller items.
- Searches for items within internal bodily cavities will never take place and will require a medical review if there is a reason to suspect that any substance/item is concealed in a body cavity.

Note: Staff must show sensitivity and respect the dignity of the person when removing clothing and recognise the added implications when removing clothing that has cultural or religious significance.

When searching a person with consent, it is beneficial to undertake the procedural search with other health assessments and interventions that require the removal of clothing. This will assist staff in maintaining the dignity of the person and may reduce the intrusive nature of a search of a person.

Example: a search of the person and the patient's personal clothing at admission can be undertaken as a component of the routine admission procedure that in many areas includes requesting the patient to shower. This allows the patient to attend to immediate hygiene needs, and for staff to perform a skin, physical injury, and physical appearance (identifying marks) assessment whilst allowing clothing to be searched thoroughly.

6.4 Patient property

When conducting a search of property staff need to be mindful of personal safety and ensure that they never reach into an area that they cannot sight visually first.

When searching property every individual item should be separated (including opening of wallets) and belongings handled to ensure all pockets or openable sections are inspected. Belongings will be handled with care and consideration. No item should be damaged by the application of force to enact the search. Electronic items are generally not opened to expose inner workings when conducting a search and may be stored in the patient property area to be returned later to the patient or returned at the time of discharge.

All property is to be listed and stored as per the [MH Management of Patient Property Procedure](#)

On reception of patient property, staff are to consider the hygiene and laundering of cloth items. Items that would benefit from laundering should be discussed with the patient and considered for laundering in accordance with the management of personal property [MHSDHS IPC Linen and Laundry Management Policy](#).

Shared environments include clinical areas that are available for all patients to use. Ideally the searching of the shared environment of a service or ward should be undertaken during cleaning of the area. During these searches all cracks, crevices, cushions and furniture should be routinely inspected.

If a non-routine search of the shared area is conducted (i.e. following loss of cutlery) staff should consider the removal of all of the patients to another area whilst the search is conducted

6.5 Patient environment

When searching a patient's environment all property and furniture should be inspected. It is beneficial to remove all patient personal property from the furniture before inspecting the furniture.

When inspecting beds staff will:

- Remove the bed linen.
- Visually inspect the pillow, mattress, and bed base for signs of damage – patients may have created a place of concealment in these furnishings.
- Physically pat or frisk the pillow and mattress.

When searching shared bedrooms, staff should consider having both patients present and searching both areas consecutively. At the conclusion of the search staff will return the room to its pre-search state.

6.6 Seizure of patient person property

Each service or ward area is to develop and review local guidelines on what items are considered unsafe for the clinical area. When deciding on appropriateness of property and items on the ward staff must refer to these local guidelines.

It is acknowledged that some patients may wish to voluntarily discard some personal items during a search. These items should not then need to be seized.

Suspected illegal substances should never be discarded, and are to be managed as per the **NMHS MH Alcohol, Illicit Substances and Non-Prescribed Substances - Management of Procedure**.

The following items may be seized under the powers vested in MHS under the *MHA 2014*, Part 11, Division 2:

- An intoxicant.
- An article, including a drug that is prescribed for the person, which may pose a serious risk to the health or safety of the person or another person.
- Personal medications.
- An article that the person conducting the search believes is a weapon, implement or an item with the potential to cause harm to the person or others and anything that is likely to provide material evidence of mental illness or a requirement for treatment under the *MHA 2014*.

6.7 Voluntary Discarding

Occasionally on reception to a MH service, a patient may wish to voluntarily discard certain personal items. Items which a patient may wish to be voluntarily discarded and is facilitated by the service include:

- Unused or used syringes.
- Items that are linked (but not involved) in a ritual of taking an intoxicant (i.e. a lighter).
- Unopened and clearly identifiable legal intoxicants (e.g. synthetic cannabinoids).
- Alcohol.
- Used personal hygiene items, or condoms.
- Food.
- Mouldy or severely damaged clothing or bedding.

- Out of date medications (prescribed or over the counter).
- Razors, blades or other sharp or potentially dangerous items.

Any item voluntarily discarded should be listed and documented as discarded. The use of the item discarded should be considered when formulating a mental state examination.

Items should be discarded into the appropriate receptacle (including sharps or clinical waste).

Alcohol can be disposed of by pouring it down a drain. If using a drain, ensure appropriate flushing afterwards to manage odour. Any other drug paraphernalia or unidentifiable intoxicant (even if suspected as legally purchasable) should be handled and disposed of as per the **NMHS MH Alcohol, Illicit Substances and Non-Prescribed Substances Management of Procedure**.

These items should be recorded and documented as seized.

Any medication for discarding should be documented on the Patient Own Medication Form and given to Pharmacy for incineration. If stored temporarily before being able to be given to Pharmacy the medication should be stored in the appropriate cupboard (including S4R or S8 cupboard). Refer to the [NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Procedure](#).

Another option for a patient who doesn't wish to discard items but still ensure safety whilst in hospital is for the patient to voluntarily hand items considered unsafe in the clinical area to a family member or personal support person. Items that are clearly hazardous, but not clearly weapons (i.e. fishing gear, garden or mechanical tools, ornamental or replica weapons) are appropriate for this option.

7. Seizure of Items

The powers to seize items can be used when a patient doesn't give voluntary consent to discard or appropriately store hazardous items. It should also be used whenever a patient is found to have a suspected illegal intoxicant, or related drug paraphernalia that may be accessed for the use of illicit substances (i.e. marijuana smoking implement).

All seizure of items must be clearly documented in both the integrated notes (including rationale for removal, associated risk assessment, and process for storage or disposal of the item) and the authorised *MHA 2014* Form 8B. A clinical incident report is to be created via [DatixCIMS](#). The original Form 8A is to be placed in the patient's medical record which copies to be supplied to:

- The patient.
- The patient's guardian or nominated person (if indicated).
- The Senior Registered Nurse – will attach a scanned copy to the [DatixCIMS](#) incident record and table the incident for review in accordance with the endorsed process for review in the service.

For sites using Digital Medical Records (DMR), store the original Form 8B in the patients Work in Progress file for later scanning in the DMR. Once scanned, the Form 8B will appear in the Legal Folder for the patients DMR. A copy of the form to be supplied to:

- The patient.
- If indicated the patient's guardian or nominated person.

- The Senior Registered Nurse – who will attach a scanned copy to the [DatixCIMS](#) incident record and table the incident for review in accordance with the endorsed process for review in the service.

The following items are processed as follows:

Alcohol – open vessels of alcohol should be disposed of with the patients consent, or placed into storage until their discharge if they do not or are unable to consent. Unopened vessels will be placed into property storage. Staff should seek approval from a senior nurse prior to disposal and all relevant information is to be documented in the patient records.

Suspected illicit substances – placed in tamper evident bag (patient own S4R bag) by two staff. Staff should write on the front panel of the bag a list of items placed in bag, the date, a patient addressograph sticker, staff's name, designation, and signature. Once this is complete the staff should then deliver the sealed bag to local hospital Security for storage and WA Police notification and destruction.

Prescribed medication (whether prescribed to the patient or not) – placed in patient own medication bag (appropriate to the schedule of the drug) and listed on the Patient Own Medication Form. If prescribed medication is seized a decision is made by the Consultant and/or with the treating team on whether it is appropriate to return this medication to the patient post discharge. If the medication is for return, the bag should be stored in the ward (in the appropriate cupboard based on the schedule of the medication). If a decision is made not to return the medication it should be placed in the appropriate bag, labelled and be delivered to Pharmacy for incineration. Refer to the [NMHS Medication Management and Administration Policy and Procedures](#) and [NMHS MH Medication: Prescribing, Administration and Management for Inpatients Policy](#) and Procedure.

An item that is clearly a weapon – weapons are documented and are signed over to local hospital Security for storage and destruction.

An item that is hazardous is clearly not a weapon but may be used as one (i.e. *fishing gear, garden tools, ornamental or replica weapons*) – ideally these should be signed over to a family member or personal support person with the consent of the patient. If this option is not available, a clinical assessment and decision on whether to return the items post discharge needs to occur. If planning on returning the items staff must list the items on the Property Sheet and store them in a designated secure area for return to the patient post discharge. If not returning the items they are required to be signed over to local hospital Security for storage and destruction.

8. Monitoring and Evaluation

Compliance with this procedure is mandatory. There are legislative obligations relating to this procedure where a statutory penalty may apply for non-compliance

Review of DatixCIMS, Consumer Feedback via the formal NMHS MH feedback processes, Consumer Satisfaction Surveys relating to the search of patient, property and environment.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every 2 years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

9. Definitions

Terms	Definitions
Routine search	Searches conducted as part of the normal routine of service provision, or part of the patient journey. For all services, routine searches should be conducted at admission, upon return from escorted or unescorted leave or ground access, prior to transfer to another MH unit, and during daily cleaning of the shared environment
Non-routine search	Searches conducted outside of the routine service provision. They are indicated only by the identification of specific risk.
Person	The body and personal clothing being worn.
Patient property	Any property belonging to the patient that is not currently on the person.
Shared environment	Patient areas that are accessed by multiple patients and are designed to be shared. e.g. dining room.
Patient environment	Patient bedroom areas or any area specifically assigned to an individual patient.
Search with consent	Searches conducted with specific verbal consent given by the patient do not require the reporting or utilising of the powers of the <i>MHA 2014</i> .
Search without consent	Searches can be conducted without specific consent under the powers of Part 11, Division 2 of the <i>MHA 2014</i> . A patient may refuse to consent to a search but still comply with the procedure of the search. This is considered a search without consent.
Voluntary discarding	When a patient requests that the service provide assistance to safely discard unwanted property.
Seizure of property	The removal by staff of patient property without consent using the powers vested in MH services under the <i>MHA 2014</i> , Part 11, Division 2.

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms

10. Related Documents

Legislation	• Mental Health Act 2014 (MHA 2014)
NMHS Policy documents	• NMHS Destruction, Discard and Disposal of Schedule 8 (S8) and Schedule 4 Restricted (S4R) Medicines Procedure
NMHS MHS Policy documents	• NMHS MH Alcohol, Illicit Substances and Non-Prescribed Substances - Management of Procedure • NMHS Medication Management and Administration Policy and Procedures • NMHS Chaperone Policy • NMHS Infection Prevention and Control Policy

	• NMHS MH Medication: Prescribing, Administration and Management for Inpatients Policy • NMHS MH Clinical Risk And Management Policy (CRAM) • NMHS MH Clinical Observation Levels Procedure • NMHS MH Management of Clinical Alerts Policy • MHSDHS Mandatory Training Policy • MHSDHS IPC Linen and Laundry Management Policy
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11. Document Control

Document Version Control			
Date	Version	Author	Notes
07/06/2018	1.2	A/Policy Officer	Review date amended to reflect MHA review. Hyperlinks repaired.
13/03/2019	2.0	Nurse Director, SFMHS	Scheduled Review: Policy revised/updated to reflect the requirements of the Mental Health Act 2014 and the Clinicians Guide to the Mental Health Act 2014, Updated references and hyperlinks, Change of Policy title, Updated template.
13/09/2022	3.0	Coordinator of Nursing SFMHS	Scheduled review, updated in accordance with overarching NMHS Policy, changed from policy to procedure for use in clinical inpatient services, template and hyperlinks updated.
25/11/2025	4.0	Operational Co Director Adult Inpatient MHSDHS	Information reviewed and updated in line with rescindment of overarching NMHS Search and Seizure Policy, MHA 2014, Safety Planning For Mental Health Consumers Policy and Management of Restrictive Practices Policy and Procedure and the MH Alcohol, Illicit Substances and Non-Prescribed Substances - Management of Procedure. State Forensics Mental Health Services included in scope.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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